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Anatomic Outcomes with Faricimab vs Aflibercept in Head-to-Head Dosing Phase of the TENAYA/LUCERNE Trials in Neovascular Age-related Macular Degeneration

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***Purpose*:** To compare early anatomic outcomes after treatment with faricimab versus aflibercept in a pooled analysis of the head-to-head dosing phase of the TENAYA/LUCERNE trials in neovascular age-related macular degeneration (nAMD).

***Design*:** TENAYA/LUCERNE (NCT03823287/NCT03823300) were identical, randomized, double-masked, active comparator-controlled phase 3 noninferiority trials.

***Participants*:** Patients aged ≥ 50 years with treatment-naïve nAMD.

***Methods*:** Patients were randomized (1:1) to intravitreal faricimab 6.0 mg up to every 16 weeks (Q16W) after 4 initial doses every 4 weeks (Q4W) or aflibercept 2.0 mg every 8 weeks (Q8W) after 3 initial doses given Q4W.

***Main Outcome Measures*:** Post hoc analyses comparing faricimab with aflibercept in terms of change in central subfield thickness (CST) from baseline and proportion of patients with an absence of subretinal fluid (SRF) and intraretinal fluid (IRF) during initial 12-week head-to-head dosing phase, when both arms received 3 injections, and time to first absence of IRF and SRF.

***Results*:** A total of 1329 patients were enrolled across TENAYA/LUCERNE (n = 665 faricimab; n = 664 aflibercept). There were greater (nominal $P < 0.0001$) reductions in adjusted mean CST from baseline with faricimab versus aflibercept at weeks 4, 8, and 12, with comparable vision outcomes. At week 12, more patients (95% confidence interval) achieved an absence of SRF (87.9% [85.4%–90.4%] vs. 79.0% [76.0%–82.1%]) and both IRF and SRF (77.2% [74.0%–80.4%] vs. 66.5% [62.9%–70.0%]) but not IRF (88.4% [86.0%–90.8%] vs. 85.0% [82.3%–87.6%]), with faricimab versus aflibercept, respectively. In patients with IRF or SRF at baseline (n = 581 faricimab; n = 591 aflibercept), the 75th percentile of time to first absence of IRF and SRF was reached at week 8 with faricimab and week 12 with aflibercept. At week 12, cumulative incidence of first-time absence of IRF and SRF was 85.5% (82.3%–88.1%) with faricimab and 75.0% (71.3%–78.3%) with aflibercept.

***Conclusions*:** Faricimab resulted in greater improvement in anatomic outcomes than aflibercept during the head-to-head dosing phase and a faster time to first absence of retinal fluid.

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Faricimab is a humanized, bispecific, immunoglobulin G monoclonal antibody that independently targets and neutralizes both angiopoietin-2 (Ang-2) and VEGF-A,[1,2] which contribute to the pathogenesis of retinal vascular diseases such as neovascular age-related macular degeneration (nAMD).[3,4] In pathologic conditions such as nAMD, Ang-2 is upregulated and plays a role in vascular homeostasis, permeability, and angiogenesis via dysregulation of the Ang-Tie2 pathway. Angiopoietin-2 also sensitizes retinal vessels to VEGF-A by stimulating pericyte detachment.[1,3] Simultaneous inhibition of both pathways with faricimab may promote vascular stabilization and disease control,[3] leading to more durable treatment in patients with nAMD.

In the phase 3 TENAYA and LUCERNE trials, faricimab at intervals of up to every 16 weeks (Q16W) was noninferior to anti-VEGF aflibercept every 8 weeks (Q8W) in terms of best-corrected visual acuity (BCVA) change from baseline to year 1 (averaged over weeks 40, 44, and 48).[2] Reductions in central subfield thickness (CST) were comparable between both treatment arms through week 48. Furthermore, at week 48, approximately 80% of faricimab-treated patients achieved every 12 weeks (Q12W) or more dosing, with approximately 45% on Q16W dosing. The potential of faricimab to extend treatment intervals while maintaining efficacy could result in meaningfully reduced treatment burden for patients with nAMD and reduced financial burden on the health care system.

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Dosing decisions for anti-VEGF agents, particularly with personalization of treatment intervals, are generally based on determination of disease activity, including visual acuity changes, new macular hemorrhage, and anatomic biomarkers, that is, OCT-based biomarkers such as the presence and changes of retinal fluid including subretinal fluid (SRF) and intraretinal fluid (IRF).[5,6] Therefore, understanding anatomic responses to faricimab treatment may help to inform future management decisions in patients with nAMD. Direct comparisons in anatomic responses between faricimab and aflibercept across the full TENAYA/LUCERNE trial periods were limited because of differences in the agents' dosing regimens per the study design.[2,7] However, during the first 8 weeks of the 12-week initial dosing phase of the trials, both arms received 3 once every 4 weeks (Q4W) doses allowing for a head-to-head comparison.[2,7] We present the early treatment response in terms of CST, IRF, and SRF from a pooled analysis up to week 12 (4 weeks after the last dose given in parallel) of the initial dosing phase in the TENAYA and LUCERNE trials to assess the impact of faricimab compared with aflibercept in patients with nAMD. Additionally, the time to first absence of IRF and SRF was assessed.

Methods

Study Design

The rationale and study design for the identical, global, phase 3, multicenter, randomized, active comparator-controlled, double-masked, parallel-group registrational TENAYA (NCT03823287) and LUCERNE (NCT03823300) trials have been described.[2,7] The study protocols were approved by appropriate regulatory authorities, applicable Institutional Review Boards, and ethics committees, and both trials were conducted following principles of Good Clinical Practice and in compliance with the Declaration of Helsinki.[2] These trials investigated the efficacy, safety, and durability of intravitreal faricimab 6.0 mg at up to Q16W intervals after 4 initial Q4W doses compared with intravitreal aflibercept 2.0 mg at Q8W intervals after 3 initial Q4W doses in treatment-naïve patients with nAMD over 2 years. The current post hoc analysis focuses on the initial head-to-head dosing phase, during which all patients received the same number of monthly doses (i.e., 3 injections). Outcomes were assessed at weeks 4, 8, and 12 (4 weeks after the last head-to-head dose). In addition, time to first absence of IRF and SRF is presented.

Participants

The inclusion and exclusion criteria for patients in the TENAYA and LUCERNE trials have been fully described.[7] In brief, patients were $\geq$ 50 years of age with active, treatment-naïve macular neovascularization secondary to age-related macular degeneration. One eye was designated as the study eye; if both eyes were eligible for inclusion, the eye with the worse BCVA at screening was selected.[7] All participants provided written informed consent.

Randomization and Masking

Patients were randomized in a 1:1 manner to receive either intravitreal faricimab 6.0 mg up to Q16W or aflibercept 2.0 mg Q8W. To preserve the masked design of this study, all patients were required to attend study visits Q4W, with sham injections being administered at nondosing visits. Further details have been previously described.[2,7]

Procedures

Dosing procedures have been previously described.[2,7] In the initial dosing phase through week 12, patients received 4 initial doses of faricimab administered Q4W (at weeks 0, 4, 8, and 12) or 3 initial doses of aflibercept Q4W (at weeks 0, 4, and 8). The head-to-head dosing phase is defined as the initial 12-week dosing phase when patients in both arms received 3 Q4W treatments (weeks 0, 4, and 8) and before the treatment regimens diverged. After the initial dosing phase, patients in the faricimab arm were assigned, based on protocol-defined disease activity assessment criteria, to faricimab Q8W, Q12W, or Q16W dosing intervals through week 60. Further treatments from week 60 to week 108 followed a treat-and-extend (T&E)-based personalized treatment interval.[2,7] Patients in the aflibercept arm received aflibercept Q8W up to week 108 after the first 3 injections.

Ocular and Anatomic Assessments

Ocular and anatomic assessments have been previously described.[2,7] Spectral-domain OCT was performed at prespecified timepoints, and resulting images were independently assessed at Duke and Vienna central reading centers to maintain the masked nature of the study and facilitate continued assessment of the nAMD course.

Outcomes

Methods and data for the primary efficacy end point of mean change in BCVA from baseline to year 1 (averaged over weeks 40, 44, and 48) have been published for the individual TENAYA and LUCERNE study populations.[2] This is an exploratory analysis of the pooled TENAYA and LUCERNE study population, focused on anatomic outcomes assessed by OCT biomarkers, such as CST, defined as distance between the internal limiting membrane and the retinal pigment epithelium, and IRF and SRF, measured in the central subfield (center 1 mm). Anatomic outcomes reported here include change from baseline in CST over the first 12 weeks, and the proportion of patients with an absence of IRF only, SRF only, or IRF and SRF over the first 12 weeks. We also report the time to first absence of IRF and SRF in those patients with IRF or SRF at baseline. Change from baseline in BCVA over the first 12 weeks is also reported.

Statistical Analysis

The intention-to-treat principle was used for all exploratory analyses, with the patients being analyzed according to the group to which they were originally randomized. All intercurrent events were analyzed as in the primary analysis. As with the primary efficacy end point analyses,[2,7] a mixed model for repeated measures (MMRM) was used for statistical analysis of BCVA and CST outcomes measured on a continuous scale over time. For the MMRM analysis, strata were defined by study

(TENAYA vs. LUCERNE) and randomization factors as described previously.[2,7] An unstructured covariance structure was used, and any missing data were imputed implicitly using this model.

For binary IRF and SRF outcomes, stratified estimation for binomial proportions was used. The proportion of patients in each treatment group and the overall difference in proportions between treatment groups were estimated using the Cochran-Mantel-Haenszel (CMH)-weighted averages over strata defined by study (TENAYA vs. LUCERNE) and randomization factors as described previously.[2,7] Kaplan−Meier estimates were generated for time to first absence of IRF and SRF through week 48 for patients with IRF or SRF at baseline. Median and 75th percentile of time to first absence of IRF and SRF through week 48 were estimated and compared between the 2 treatment arms. Hazard ratios were estimated by Cox regression. Both Kaplan−Meier and Cox regression analyses were adjusted for study (TENAYA vs. LUCERNE) randomization factors.[2,7] For these exploratory analyses, all P values were nominal and unadjusted for multiplicity.

Results

Baseline Demographics and Characteristics

In total, 1329 patients were enrolled across the TENAYA and LUCERNE trials and randomized 1:1 to receive faricimab (n = 665) or aflibercept (n = 664).[2] Baseline demographics have been previously published and were generally well balanced across treatment arms.[2,8] In brief, BCVA (standard deviation) (faricimab, 60.0 [13.3] letters; aflibercept, 60.2 [13.1] letters), CST (faricimab, 356.8 [122.1] μm; aflibercept, 357.5 [119.4] μm), the number (proportion) of patients with presence of IRF (faricimab, 288 [43.3%]; aflibercept, 311 [46.8%]), and the number (proportion) of patients with SRF (faricimab, 437 [65.7%]; aflibercept, 447 [67.3%]) were comparable between treatment arms at baseline.[8] In the faricimab and aflibercept arms, 88.4% and 89.7% of patients, respectively, had IRF or SRF at baseline.

Visual and Anatomic Changes from Baseline

There were comparable adjusted mean gains in BCVA (95% confidence interval [CI]) from baseline with faricimab and aflibercept at week 4 (+4.6 [+4.0 to +5.2] and +4.0 [+3.4 to +4.6] letters, respectively), week 8 (+5.8 [+5.1 to +6.4] and +5.2 [+4.5 to +5.8] letters, respectively), and week 12 (+6.7 [+6.1 to +7.4] and +5.9 [+5.2 to +6.5] letters, respectively) (Fig 1A). There were greater (all nominal $P < 0.0001$) adjusted mean reductions in CST (95% CI) from baseline with faricimab than aflibercept at week 4 (−128.8 [−133.0 to −124.6] vs. −115.0 [−119.2 to −110.8]; difference [95% CI] −13.8 [−19.7 to −7.8] μm), week 8 (−140.3 [−144.2 to −136.4] vs. −127.7 [−131.6 to −123.8]; difference [95% CI] −12.6 [−18.2 to −7.0] μm), and week 12 (−145.4 [−149.1 to −141.8] vs. −133.0 [−136.7 to −129.3]; difference [95% CI] −12.5 [−17.7 to −7.3] μm) (Fig 1B).

Absence of Retinal Fluid

When looking at resolution of fluid, absence of IRF only, SRF only, and IRF and SRF were also evaluated over the initial head-to-head dosing phase (Fig 2). In both treatment arms, the baseline proportion of patients with absence of IRF only (faricimab, 55.9%; aflibercept, 52.8%), SRF only (faricimab, 33.5%; aflibercept, 32.0%), and IRF and SRF (faricimab, 11.6%; aflibercept, 10.3%) increased after initiating treatment.

The proportion of patients with an absence of IRF was comparable between faricimab and aflibercept arms (respective CMH-weighted estimates [95% CI] of 88.1% [85.7%−90.4%] vs. 84.1% [81.4%−86.8%] at week 4, 88.0% [85.6%−90.4%] vs. 84.5% [81.8%−87.1%] at week 8, and 88.4% [86.0%−90.8%] vs. 85.0% [82.3%−87.6%] at week 12). The proportion of patients with an absence of SRF was greater (all nominal $P < 0.05$) with faricimab than in the aflibercept arm (respective CMH-weighted estimates [95% CI] of 69.3% [65.9%−72.7%] vs. 60.2% [56.6%−63.9%] at week 4, 82.2% [79.3%−85.1%] vs. 75.0% [71.7%−78.3%] at week 8, and 87.9% [85.4%−90.4%] vs. 79.0% [76.0%−82.1%] at week 12). Differences in CMH-weighted estimates (95% CI) for SRF between faricimab and aflibercept were 9.1% (4.1%−14.1%; nominal $P = 0.0006$) at week 4, 7.2% (2.8%−11.5%; nominal $P = 0.0018$) at week 8, and 8.9% (4.9%−12.8%; nominal $P < 0.0001$) at week 12.

The proportion of patients with an absence of both IRF and SRF was greater (all nominal $P < 0.05$) with faricimab than aflibercept (respective CMH-weighted estimates [95% CI] of 60.2% [56.5%−63.9%] vs. 49.4% [45.6%−53.1%] at week 4, 72.4% [69.0%−75.8%] vs. 62.4% [58.7%−66.1%] at week 8, and 77.2% [74.0%−80.4%] vs. 66.5% [62.9%−70.0%] at week 12). Differences in CMH-weighted estimates (95% CI) for absence of IRF and SRF between faricimab and aflibercept were 10.8% (5.5%−16.0%; nominal $P < 0.0001$) at week 4, 10.0% (5.0%−15.0%; nominal $P = 0.0001$) at week 8, and 10.7% (5.9%−15.5%; nominal $P < 0.0001$) at week 12.

Time to First Absence of Retinal Fluid

In patients who had IRF or SRF at baseline (n = 581 faricimab; n = 591 aflibercept), median and 75th percentile of time to first absence of retinal fluid (IRF and SRF) occurred during the 12-week head-to-head dosing phase (Fig 3). The median time to first absence of retinal fluid with faricimab was 4 weeks shorter than with aflibercept (median 4 vs. 8 weeks) and was achieved with fewer injections than aflibercept (median [minimum−maximum] number of injections 1 [0−1] for faricimab vs. 2 [0−2] for aflibercept). The 75th percentile of the time to first absence of IRF and SRF was achieved at week 8 in the faricimab arm after a median (minimum to maximum) of 2 (0−2) injections; the same percentile in the aflibercept arm was not achieved until week 12, after a median (minimum to maximum) of 3 (0−3)

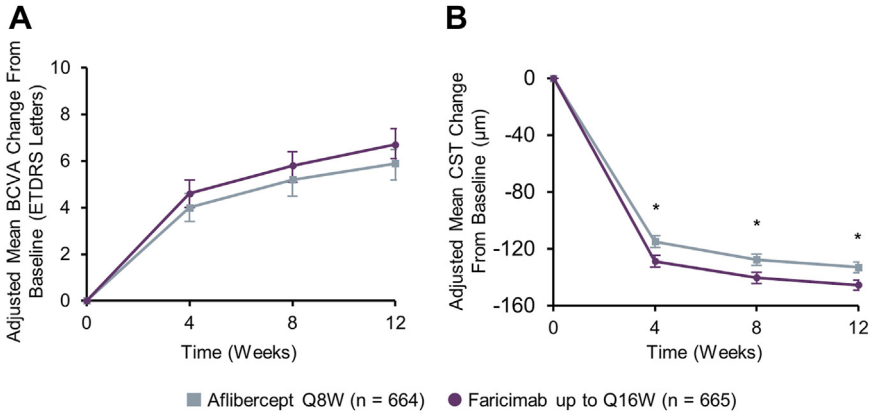

Figure 1. Adjusted mean change from baseline in **(A)** BCVA and **(B)** CST during the 12-week head-to-head dosing phase (weeks 0–12). Results are based on an MMRM analysis of the intention-to-treat population. The CST is measured as ILM-RPE, as graded by a central reading center. Treatment policy strategy and hypothetical strategy were applied to non–COVID-19–related and COVID-19–related intercurrent events, respectively. *P* values are nominal and not adjusted for multiplicity; no formal statistical conclusion should be made based on the *P* values. Shown are 95% CIs. *Nominal $P < 0.0001$ for faricimab versus aflibercept. BCVA = best-corrected visual acuity; CI = confidence interval; COVID-19 = Coronavirus Disease 2019; CST = central subfield thickness; ILM = internal limiting membrane; MMRM = mixed model for repeated measures; Q8W = every 8 weeks; Q16W = every 16 weeks; RPE = retinal pigment epithelium.

injections. At week 12, cumulative incidence was 85.5% (95% CI: 82.3%–88.1%) in the faricimab arm and 75.0% (95% CI: 71.3%–78.3%) in the aflibercept arm (Fig 3).

During the first year of treatment, patients in the faricimab arm had a 47% increase in the chance of achieving first absence of retinal fluid compared with the aflibercept arm (hazard ratio, 1.47 [95% CI, 1.24–1.73]; log-rank nominal $P < 0.0001$).

Safety

Faricimab was well tolerated with a safety profile comparable to aflibercept: The full safety profiles from TENAYA and LUCERNE have been published previously.[2]

Discussion

In the TENAYA and LUCERNE trials, faricimab administered up to Q16W was found to be noninferior to aflibercept administered Q8W in terms of change from baseline in BCVA.[2] However, differences in dosing regimens between faricimab and aflibercept limited the comparability of anatomic responses during the maintenance period. In this post hoc analysis of the 12-week head-to-head dosing phase of the pooled TENAYA and LUCERNE trials, in which both arms received the same number of Q4W initial treatments, faricimab resulted in both a greater and faster improvement in anatomic outcomes than aflibercept. This included a greater (nominal $P < 0.0001$) reduction in mean CST from baseline over the first 12 weeks, a higher proportion of patients achieving an absence of SRF (all nominal $P < 0.05$) and retinal fluid (IRF and SRF; all nominal $P < 0.05$) through week 12, and a faster (nominal $P < 0.0001$) time to achieve first absence of retinal fluid in those patients with IRF or SRF at baseline.

In this analysis, absence of retinal fluid was rapidly induced with faricimab treatment: Approximately 60% of patients had absence of retinal fluid by week 4, increasing to approximately 77% by week 12. Rapid control of retinal fluid in the initial head-to-head dosing phase of the present study suggests that faricimab had an early impact on controlling nAMD disease activity. We hypothesize the early disease control observed with faricimab treatment is driven by the vascular stabilizing effects of Ang-2 inhibition beyond anti–VEGF-A alone, which is supported by preclinical studies demonstrating reduced choroidal neovascularization leakage, neovascularization, and inflammation in a mouse model of spontaneous choroidal neovascularization.[1] Furthermore, the phase 2 CANDELA trial, which compared aflibercept 8.0 mg with aflibercept 2.0 mg, and the HARBOR trial, which compared ranibizumab 2.0 mg with ranibizumab 0.5 mg,[9,10] showed that simply increasing the dose of anti-VEGF does not result in further improved anatomic outcomes in the first 12 weeks of treatment.

Despite the observed improvement in anatomy with faricimab versus aflibercept, vision gains were not significantly different between treatments. This highlights the complexity in evaluating the correlation between structure and function in a multifactorial disease such as nAMD.[11] It is well recognized that VA can be affected by factors beyond the control of exudative activity, such as retinal structural abnormalities. The clinical implications of the observed difference in anatomy between faricimab and aflibercept

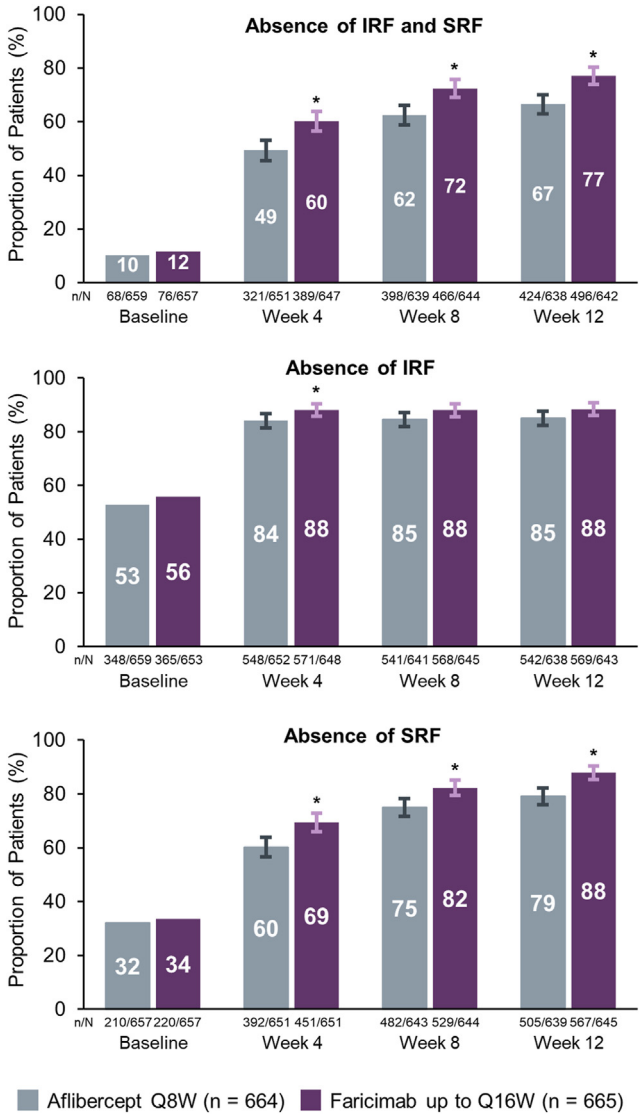

Figure 2. Proportion of patients who achieved absence of IRF, SRF, or IRF and SRF during the head-to-head dosing phase (weeks 0–12). *Nominal $P < 0.05$ versus aflibercept Q8W; nominal $P > 0.05$ where no asterisk is shown. Treatment policy strategy and hypothetical strategy were applied to non–COVID-19–related and COVID-19–related intercurrent events, respectively. Weighted proportions were estimated for the intention-to-treat population using the CMH test method. All values except at baseline are CMH-weighted estimates, which are based on CMH test stratified by baseline BCVA ($\geq$ 74, 73–55, $\leq$ 54 letters), baseline LLD ($<$ 33, $\geq$ 33 letters), region (United States and Canada vs. the rest of the world), and study (TENAYA vs. LUCERNE). The IRF and SRF are as measured in the central subfield (center 1 mm). The 95% CIs are shown, and estimates $<$ 0% or $>$ 100% are imputed as 0% or 100%, respectively. BCVA = best-corrected visual acuity; CI = confidence interval; CMH = Cochran-Mantel-Haenszel; COVID-19 = coronavirus disease 2019; IRF = intraretinal fluid; LLD = low-luminance deficit; Q8W = every 8 weeks; Q16W = every 16 weeks; SRF = subretinal fluid.

may be most relevant in the 33.5% of patients who still had retinal fluid after 3 loading doses of aflibercept. However, the study was not designed or powered to demonstrate differences between or within subgroups, and further studies are needed to evaluate this possibility. Regardless, early anatomic response may be associated with rapid extension and durability. OCT-based fluid evaluation generally offers an objective way to evaluate control of neovascularization activity. With fluid currently being the best biomarker for disease activity, early absence of retinal fluid is considered an indication that the disease is inactive or under control. Because both clinicians and patients typically aim to extend treatment intervals as soon as possible, patients who achieve absence of fluid earlier may be less likely to be undertreated and more likely to extend treatment intervals sooner, helping reduce the treatment burden.

Decrease in CST was one of the disease activity criteria used to assign patients to extended dosing regimens in the TENAYA and LUCERNE trials. Therefore, faricimab-treated patients who show early control of nAMD (i.e., rapid reductions in retinal fluid) may be more likely to achieve early extension of their dosing intervals, because fewer injections of faricimab may be necessary to achieve and maintain disease control. The findings of this study also demonstrate that a greater proportion of faricimab-treated patients achieved an absence of retinal fluid compared with aflibercept, which may translate into more patients extending their treatment interval sooner.

Data from 1- and 2-year anatomic outcomes of the TENAYA/LUCERNE trials—with sustained and stable reduction in CST and approximately 80% of faricimab-treated patients on fixed more than or equal to Q12W dosing from weeks 20 and 24 through 48,[2] indicating rapid extension, and approximately 71% maintaining more than or equal to Q12W dosing without reduction to Q8W during the T&E phase through week 112[12]—suggest that early control of nAMD disease activity was likely maintained over the 2 years. The maintenance of the extended dosing interval also may be driven by the vascular stabilizing effects of Ang-2 inhibition, which remains suppressed $\geq$ 16 weeks after faricimab administration.[13,14] Reduced treatment frequency may help avoid patient undertreatment, while also reducing the strain on caregivers and healthcare providers, as demonstrated with other T&E-based approaches.[15]

Limitations

A limitation of this analysis is its post hoc exploratory nature, and thus the results should be interpreted with care. Also, direct comparisons between faricimab and aflibercept were not available after 12 weeks because faricimab was given in an up to Q16W regimen whereas aflibercept was given in a fixed Q8W regimen, per the study design.

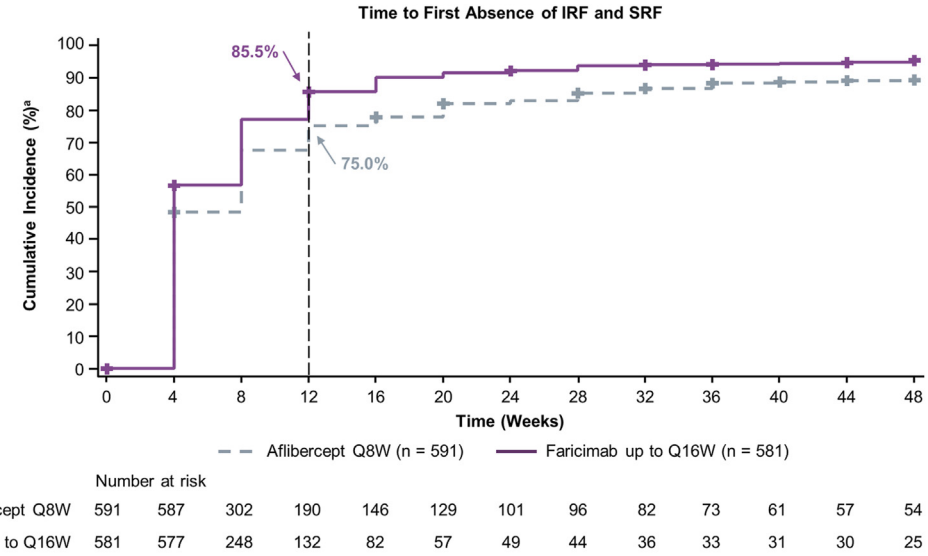

Figure 3. Time to first absence of IRF and SRF in patients with IRF or SRF at baseline. Summaries of time to first absence of IRF and SRF are Kaplan–Meier estimates, with the time variable defined as the target visit week. [a]The IRF and SRF are as measured in the central subfield (center 1 mm). Event is defined as the first absence of IRF and SRF after randomization date. Patients with absence of IRF and SRF at baseline were excluded from the analysis. + Censored observation. IRF = intraretinal fluid; Q8W = every 8 weeks; Q16W = every 16 weeks; SRF = subretinal fluid.

Conclusions

Faricimab exerted rapid control over fluid biomarkers of underlying nAMD activity, and a greater (all nominal $P < 0.05$) proportion of patients treated with faricimab achieved an absence of retinal fluid compared with aflibercept during the 12-week head-to-head dosing phase of TENAYA/LUCERNE. When treated with faricimab, patients with IRF or SRF at baseline achieved first absence of retinal fluid faster than with aflibercept. Dual inhibition of Ang-2 and VEGF-A with faricimab may allow for more rapid anatomic improvement than VEGF blockade alone. This more rapid improvement in anatomic outcomes with faricimab may result in early treatment interval extension, potentially leading to a reduction in treatment burden.

Roche Data Sharing Statement

For up-to-date details on Roche's Global Policy on the Sharing of Clinical Information and how to request access to related clinical study documents, see https://go.roche.com/data_sharing

Anonymized records for individual patients across more than one data source external to Roche cannot, and should not, be linked due to a potential increase in risk of patient re-identification.

Requests for the data underlying this publication require a detailed, hypothesis-driven statistical analysis plan that is collaboratively developed by the requestor and company subject matter experts. Direct such requests to Roche for consideration.

Footnotes and Disclosures

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HUMAN SUBJECTS: Human subjects were included in this study. The study protocols were approved by appropriate regulatory authorities, applicable institutional review boards, and ethics committees and both trials were conducted following principles of Good Clinical Practice. IRB approval was granted by each study site participating in the TENAYA/LUCERNE trial (referred to in reference 2). All research adhered to the tenets of the Declaration of Helsinki. All participants provided informed consent.

No animal subjects were used in this study.

Author Contributions:

Conception and design: Cheung, Margaron, Souverain, Yang, Guymer

Data collection: Souverain, Yang

Analysis and interpretation: Cheung, Lim, Priglinger, Querques, Margaron, Patel, Souverain, Willis, Yang, Guymer

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Overall responsibility: Cheung, Lim, Priglinger, Querques, Margaron, Patel, Souverain, Willis, Yang, Guymer

Abbreviations and Acronyms:
Ang-2 = angiopoietin-2; **BCVA** = best-corrected visual acuity; **CI** = confidence interval; **CMH** = Cochran-Mantel-Haenszel; **CST** = central subfield thickness; **IRF** = intraretinal fluid; **MMRM** = mixed model for repeated measures; **nAMD** = neovascular age-related macular degeneration; **Q8W** = every 8 weeks; **Q12W** = every 12 weeks; **Q16W** = every 16 weeks; **SRF** = subretinal fluid; **T&E** = treat-and-extend.

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